

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Mysimba (Naltrexone 8mg / Bupropion 90mg) prolonged-release tablets for the treatment of obesity

Patient Group Direction, version 003, issued 11 September 2026. Supersedes Version 002, 10 September 2026.

Summary of NICE / NICE CKS Guidelines for obesity

Overview

Obesity is a chronic disease affecting a significant proportion of the population. Clinical assessment should include measurement of weight, height and calculation of body mass index (BMI), waist circumference, and assessment of cardiovascular risk.

- **Management approach:** The primary management of obesity involves lifestyle intervention with a reduced-calorie diet and increased physical activity. Pharmacological interventions, including Mysimba, should be considered as adjunctive therapy in patients who have not achieved adequate weight loss through lifestyle measures alone.

Pharmacological treatment

Mysimba (naltrexone/bupropion combination) is a second-line pharmacological option for weight management in adults with obesity who have not achieved adequate weight loss with lifestyle intervention alone.

- **Indication:** Adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with initial BMI ≥ 30 kg/m² or ≥ 27 kg/m² in the presence of at least one weight-related comorbidity.
- **Duration of treatment:** Treatment should be reviewed at 16 weeks. If weight loss of at least 5% has not been achieved by 16 weeks, Mysimba should be discontinued.

Patient Group Direction

for the supply/administration of

Mysimba (Naltrexone 8mg / Bupropion 90mg) prolonged-release tablets

for the treatment of

obesity

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	- Pharmacist registered and practicing with the GPhC - Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. - All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines - All users must previously have used PGDs to supply medication - Must work in compliance with the SOPs of their own employer - All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	- All users must be up to date with CPD requirements and appraisal - All users must be up to date with MHRA and safety alerts with regards to medical products - All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Adjunct to a reduced-calorie diet and increased physical activity for weight management in adults with initial BMI ≥ 30 kg/m ² or ≥ 27 kg/m ² with at least one weight-related comorbidity (such as type 2 diabetes, hypertension, dyslipidaemia, or sleep apnoea).
Inclusion criteria	Age 18 years and over BMI ≥ 30 kg/m² or ≥ 27 kg/m² with at least one weight-related comorbidity Documented failed weight loss attempt through lifestyle intervention (reduced-calorie diet and physical activity) for at least 3 months Able to provide informed written consent Not currently pregnant or breast-feeding; females of childbearing potential should use effective contraception Resting blood pressure <140/90 mmHg (if $\geq 140/90$ mmHg, refer to GP for management before initiating Mysimba)
Exclusion criteria	Uncontrolled hypertension (blood pressure $\geq 140/90$ mmHg) or history of significant cardiovascular disease Seizure disorders or history of seizures or head trauma with loss of consciousness CNS tumour or history of CNS tumour

	Abrupt discontinuation of alcohol or benzodiazepines, or concurrent use of benzodiazepines or alcohol withdrawal Current opioid use or opioid dependence; do not use within 7-10 days of opioid discontinuation Use of monoamine oxidase inhibitors (MAOIs) or within 14 days of MAOI discontinuation History of eating disorders (bulimia nervosa or anorexia nervosa) Severe hepatic impairment (Child-Pugh Class C) End-stage renal failure (eGFR <15 mL/min/1.73m²) Any history of bipolar disorder Current depression, suicidal ideation, or any history of suicide attempt Currently taking any other product containing bupropion (for example Zyban) or naltrexone Known hypersensitivity to naltrexone, bupropion, or any excipients Angle-closure glaucoma
Cautions	Hypertension: Monitor blood pressure regularly (at least monthly initially). Bupropion may elevate blood pressure; discontinue if sustained elevation occurs. Hepatic impairment: Use with caution in mild to moderate hepatic impairment. Avoid in severe impairment. Renal impairment: Use with caution in moderate to severe renal impairment; dose adjustment may be required. Elderly patients: Use with caution; dose adjustment may be necessary due to age-related changes in metabolism. Suicidal ideation and behaviour: Monitor for mood changes, depression, anxiety, and suicidal thoughts, particularly in the first weeks of treatment and following any dose adjustment. Seizure threshold: Bupropion lowers seizure threshold; avoid concurrent use of medications that lower seizure threshold. Caution in patients with CNS conditions. Blood glucose: May affect glucose control in diabetic patients; monitor blood glucose closely and adjust antidiabetic medication if necessary. Concurrent medications: Potential for drug interactions with medications metabolised by CYP2D6, antidepressants, and other CNS-active drugs; review all medications before initiating. Angle-closure glaucoma risk: Bupropion may increase intraocular pressure; caution in patients at risk of angle-closure glaucoma. Tachycardia: Monitor heart rate; bupropion may increase heart rate or cause tachycardia.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be

	accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.
--	---

Details of the medicine

Name, form and strength of medicine	Mysimba tablets containing naltrexone hydrochloride 8mg and bupropion hydrochloride 90mg (prolonged-release)
Legal category	POM (Prescription Only Medicine)
Storage	Store below 30°C. Keep in original container.
Route/method of administration	Oral. Swallow tablets whole. Do not crush, chew, or divide tablets. Can be taken with or without food (avoid high-fat meals).
Dose and frequency	Titration over 4 weeks: • Week 1: One tablet once daily in the morning • Week 2: One tablet in the morning and one tablet in the evening • Week 3: Two tablets in the morning and one tablet in the evening • Week 4 onwards (maintenance): Two tablets in the morning and two tablets in the evening (maximum 4 tablets per day, 2 twice daily) After titration, the dose should be maintained at 4 tablets daily (2 tablets twice daily) unless dosage adjustment is required.
Quantity to be administered and/or supplied	Up to 120 tablets (28-day supply at full maintenance dose of 4 tablets daily). Quantity may be adjusted based on individual patient circumstances and treatment duration.
Maximum or minimum treatment period	Review at 16 weeks of treatment: the pharmacist weighs the patient and records the result. Discontinue if less than 5% of initial body weight has been lost. Maximum treatment period under this PGD: 16 weeks (the titration month and three maintenance months). Continuation beyond 16 weeks is by prescription from the GP or a specialist prescriber, not under this PGD. Follow-up: weight, blood pressure and pulse at 4 weeks (end of titration) and at 16 weeks; ask about mood, suicidal thoughts, seizures and symptoms of raised blood pressure at every supply, and stop and refer if any is present.
Adverse effects	Very common ($\geq 1/10$): Nausea, constipation, headache, insomnia Common ($\geq 1/100$ to $< 1/10$): Vomiting, dry mouth, dizziness, anxiety, tremor, upper abdominal pain, tachycardia, hypertension, decreased appetite, rash, pruritus Uncommon ($\geq 1/1,000$ to $< 1/100$): Seizures, fever, agitation, abnormal dreams, palpitations, sweating, dyspepsia Serious adverse effects (report immediately): Seizures, severe hypertension, hepatotoxicity (jaundice, elevated liver enzymes), severe allergic reactions (including angioedema), suicidal ideation or

	behaviour, severe mood changes
--	--------------------------------

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with Mysimba. Provide written information regarding the dose titration schedule, lifestyle modifications (reduced-calorie diet and physical activity), and emergency contact details.
Follow-up advice to be given to patient or carer	Swallow tablets whole; do not crush, chew, or divide them. Follow the dose titration schedule carefully over the first 4 weeks; do not increase the dose faster than prescribed. Take the medication as prescribed even if you do not see immediate results; weight loss may take several weeks to become apparent. Maintain a reduced-calorie diet and engage in regular physical activity as directed by your healthcare provider or dietitian. Report any mood changes, depression, anxiety, suicidal thoughts, or unusual behaviour immediately to your healthcare provider or emergency services. Monitor your blood pressure regularly at home if possible; report any persistent elevation to your GP. Do not consume excessive alcohol as it may increase the risk of seizures and adverse effects.

	Avoid high-fat meals as they may affect absorption; take with water. Do not stop the medication abruptly; discuss discontinuation with your healthcare provider. Inform all healthcare providers (including dentists) that you are taking Mysimba, as it may interact with other medications. If you experience persistent insomnia, headache, nausea, or constipation, discuss management options with your healthcare provider. Attend all follow-up appointments as scheduled, particularly the review at 16 weeks to assess weight loss and tolerability. Report any allergic reactions (rash, itching, swelling) or chest pain to your healthcare provider or seek emergency care immediately.
--	---

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Weight management in obesity
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
11/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

A maximum treatment period is stated: 16 weeks under this PGD, continuation by prescription only. The previous version left the maximum to be 'determined based on continued clinical benefit', which is not a maximum. The 16-week weight review is assigned to the pharmacist and a follow-up schedule added.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Exclusion criteria: 'Bipolar disorder or major depressive disorder with suicidal ideation' read as excluding bipolar disorder only where suicidal ideation was present. The SmPC (4.3) contraindicates any history of bipolar disorder. Split into two exclusions.

Exclusion added: any other product containing bupropion (for example Zyban) or naltrexone, which the SmPC contraindicates and the previous version did not mention.

Signed on behalf of Get Real Health

Version v003, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.